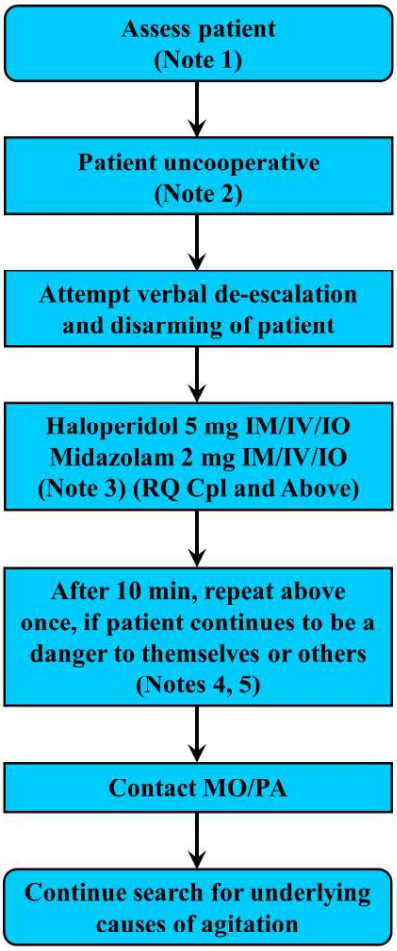
4.4 Hostile / Violent Patient – Class B

Indication:
- *Uncontrollable adult patient threatening to harm themselves, others or otherwise jeopardizing safety.*



Notes:

1. *Assess for medical causes of agitation including: Hypoglycemia; Hypoxia; Drug overdose/poisoning; Infection; Intracranial lesion; Others.*
2. *Agitation or uncooperativeness alone is not grounds for medical intervention. The need to intervene should be evaluated given appropriate consideration of the situation, the patient's need for care and the degree of risk/threat presented. The provider's safety is a priority. Any attempt at de-escalation and disarming should utilize appropriate/available resources (e.g. Military Police).*
3. *Chemical restraint should only be considered when all other means of de-escalation have failed. Ideally in highly uncooperative patients, there should be 5 people to hold patient in place for IM injection; one for the head and one for each extremity. Haloperidol and Midazolam are compatible when combined in the same syringe.*
4. *Max dose – Haloperidol 10 mg IM/IV/IO, Midazolam 4 mg IM/IV/IO. Monitor for adverse reactions to medications: Haloperidol – dystonic reactions (muscle spasms) may require treatment with Diphenhydramine 50 mg IV/IM q6h; Midazolam and Haloperidol may cause respiratory depression requiring ventilatory support.*
5. *If chemical restraint is unsuccessful, patients may also be physically restrained with non-constrictive padded items around each extremity and pelvis. Ensure patient is restrained face up on their back and continuously monitored. Physical restraint should be performed only as a last resort and by qualified personnel.*